

Lesson 1: Skilled Nursing Facilities (SNF) and the Patient Driven Patient Model (PDPM)

The Skilled Nursing Facilities' (SNF) Prospective Payment System (PPS), called the Patient- Driven Patient Model (PDPM), became effective in 1998. The payment rates include routine care, ancillary services, and capital costs. All costs associated with covered SNF services furnished to Medicare Part A beneficiaries. Consolidated billing requires the SNF to pay for outpatient services that a resident may receive from outside vendors. Therefore, outside vendors may not bill Medicare--they must bill the SNF instead.

In this lesson we will delve into the SNF healthcare setting and explore the payment systems and coding considerations.

Contemplate

Are the ICD-10-CM diagnosis code(s) assigned the only item that impacts the final reimbursement for an SNF stay?

As an example, consider the case of Mrs. Smith, who was admitted to the SNF to receive occupational and physical therapy for the right hemiplegia and dysphagia she had from her recent cerebral infarction. When Sarah coded Mrs. Smith's SNF stay, she captured the hemiplegia as the primary diagnosis but neglected to code the dysphagia. The dysphagia is a Speech Language Pathology (SLP) comorbidity under PDPM. Leaving off this ICD-10-CM diagnosis code could negatively impact the final reimbursement for Mrs. Smith's stay. Several pieces of information are needed to determine the final SNF reimbursement through PDPM.

The Healthcare Setting: Skilled Nursing Facilities

SNFs can be freestanding facilities, hospital-based units, or swing beds in acute-care hospitals. A SNF can be another stopping point for patients for medical care after they get discharged from the hospital and do not need an intensive level of care before they can live on their own.

In acute care hospitals, swing beds are beds that may be used for both acute inpatient care and skilled nursing care. Typically, these acute-care hospitals are small, rural hospitals or critical access hospitals. CMS must approve the dual use of the beds.

Under PDPM, SNFs are paid according to a per diem PPS base on case mix adjusted payment rates. Major elements of the SNF PPS include rate, coverage, transition, and consolidated billing.

1. Minimum Data Set (MDS)

The MDS is an extensive database of clinical data, including comprehensive assessments and treatment plans, which must be reported within prescribed time frames. Services that are outside of the scope of the treatment plan may be excluded from payment.

2. Patient Driven Payment Model (PDPM)

Effective October 1, 2019, the SNF reimbursement classification system called PDPM was adopted. It was designed to assign residents to payment categories based on individual patient characteristics rather than reimbursing facilities based on therapy minutes as the previous Resource Utilization Groups (RUGs) did.

2. Defining PDPM

CMS' new patient driven payment model (PDPM) is a case-mix classification system for Medicare SNF patients. The case mix group is determined by certain elements on the resident assessment instrument known as the Minimum Data Set (MDS), much like the Inpatient Rehabilitation Facility Patient Assessment Instrument (IRF-PAI).

PDPM is focused on the patient rather than the volume of services (therapies primarily) provided; reduces administrative burden on providers; improves SNF payments to currently underserved beneficiaries without increasing total MCR payments. It is more patient-driven than the previous Resource Utilization Groups (RUG-IV) system was.

The PDPM model uses six components to determine the per diem rate for SNF resident stays:

1. Nursing
2. Physical therapy (PT)
3. Occupational therapy (OT)
4. Speech-language pathology services (SLP)
5. Nontherapy ancillary services and supplies (NTA)
6. Non-case-mix category (includes room and board services)

Coding Considerations

SNF patients are first classified into a clinical category based on the primary diagnosis for the SNF stay. ICD-10-CM codes are mapped to a PDPM clinical category, which may be adjusted if the patient received a surgical intervention during the preceding hospital stay associated with that diagnosis. Based on the primary

diagnosis and presence of a surgical category, one out of a possible ten clinical categories are assigned:

- Major Joint Replacement or Spinal Surgery
- Non-Surgical Orthopedic/Musculoskeletal
- Orthopedic Surgery (Except Major Joint Replacement or Spinal Surgery)
- Acute Infections
- Medical Management
- Cancer
- Pulmonary
- Cardiovascular and Coagulations
- Acute Neurologic
- Non-Orthopedic Surgery

It is imperative to identify and properly code the ICD-10-CM codes that best reflect the patient's reason for Skilled Medicare Coverage upon admission to the SNF to maintain compliant coding.

Optional Reading: To view a mapping of the ICD-10 diagnosis and/or surgical categories used to classify a SNF resident into each of the ten clinical categories, review the following page provided by the Centers for Medicare & Medicaid Services: "[Patient Driven Payment Model](#)".



1. Once on the CMS website PDPM page, scroll to the bottom to the heading "PDPM Resources".
2. Select the item titled: FY 2021 PDPM ICD-10 Mappings (ZIP).
3. You will see this is a ZIP file so once you open it, select the spreadsheet titled: PDPM_ICD10_Mappings_FY2021.
4. This spreadsheet includes a tab titled: Clinical_Categories_By_DX.
5. This sheet lists the ICD-10-CM codes and the associated clinical category.



Required Reading: Read the "Patient-Driven Payment Model (PDPM)" and "Skilled Nursing Facility Value-Based Purchasing Program" sections, 7th Ed. pages 92-100 or 8th Ed. pages 118-132 in Chapter 6 of *Principles of Healthcare Reimbursement and Revenue Cycle Management*.

Check Your Understanding

Check your understanding of the lesson content using the practice activity below. There are six true/false questions designed to test your knowledge. When ready to submit each answer, select the checkmark submit button to verify that you are correct.

1. PDPM became effective in 2019.

☒ True

☐ False



Correct! CMS' new patient driven payment model (PDPM) became effective October 1, 2019.

Continue

2. The previous SNF PPS was called PDGM.

☒ True

☐ False



Incorrect. The previous SNF PPS was called Resource Utilization Group, Version IV (RUG-IV).

Continue

3. The PDPM model uses six components to determine the per diem rate for SNF resident stays.

☒ True

☐ False



Correct! The PDPM model uses six components to determine the per diem rate for SNF resident stays - they are Nursing, Physical therapy (PT), Occupational therapy (OT), Speech-language pathology services (SLP), Nontherapy ancillary services and supplies (NTA), and Non-case-mix category (includes room and board services).

Continue

4. PDPM is focused on the patient rather than the volume of services.

☒ True

☐ False



Correct! PDPM is focused on the patient rather than the volume of services.

Continue

5. Speech-language pathology services is one of the components used to determine the per diem rate for SNFs.

- ☒ True
☐ False



Correct! Speech-language pathology services is one of the components used to determine the per diem rate for SNFs.

Continue

6. ICD-10 diagnosis and/or surgical category is used to classify a SNF resident into each of the 10 clinical categories

- ☒ True
☐ False



Correct! ICD-10 diagnosis and/or surgical category is used to classify a SNF resident into each of the 10 clinical categories.

Start Over

Module 2 : Medicare Payment systems and Physician Reimbursement

Lesson 2: Home Health Care and the Patient-Driven Groupings Model (PDGM)

The current prospective payment system (PPS) for Home Health Agencies (HHAs) is known as the Patient-Driven Groupings Model (PDGM). It became effective for home health periods of care starting on and after January 1, 2020, replacing the former Home Health reimbursement system, which was known as the Home Health Resource Group (HHRG) system.

In this lesson we will delve into the Home Health setting and explore the payment system and coding considerations.

To Do:

1. Study the lesson content below
2. Check your understanding using the activity at the end of the presentation

Contemplate

Why is it important for a coding professional to understand the impact coding has on the Home Health PDGM payment system?

As an example, consider the case of Mr. Jones, who was receiving home health services for his low back pain per the provider's documentation. The home health coder assigned ICD-10-CM M54.5 for the low back pain to the 30-day period for the services. However, PDGM classifies this as a vague code as it is not assigned to a clinical category. Therefore, the claim was returned from the payer for a more specific code which delayed payment. Coding professionals need to understand the nuances of PDGM coding so that if clarification is needed, the provider can be queried *prior* to coding. This helps assure the claim is processed without delay.

The Healthcare Setting: Home Health Care

Beginning in January of 2020, the Centers for Medicare Services (CMS) implemented the Patient-Driven Groupings Model (PDGM) which is a PPS system. Similar to the SNF PDPM, PDGM depends more on clinical characteristics and other patient information to put home health periods of care into payment categories and eliminates the use of therapy service thresholds.

The payment unit for PDGM is a 30-day period versus the 60-day period that was used in the past with the Home Health Resource Group (HHRG) reimbursement system. However, the timeframe to re-certify eligibility and review the home health plan of care remains at 60-days as it was under HHRG.

Defining PDGM

How the PDGM works:

Case- Mix Variables

There are five main case-mix variables:

1. Admission Source (subgroups of community or institutional admission source)
2. Timing of the 30-day period (subgroups of early or late)
3. Clinical Grouping (12 subgroups) – determined by the principal diagnosis and indicates the primary reason the patient is receiving the home health services.
 - Musculoskeletal rehabilitation
 - Neuro/stroke rehabilitation
 - Wounds
 - Medication Management, Teaching, and Assessment (MMTA) – surgical aftercare
 - MMTA – cardiac and circulatory
 - MMTA – endocrine
 - MMTA – gastrointestinal tract and genitourinary system
 - MMTA – infectious disease, neoplasms, and blood-forming diseases
 - MMTA – respiratory
 - MMTA – other
 - Behavioral health
 - Complex nursing interventions
 - *Example:* The patient's principal diagnosis is documented as stage 3 pressure ulcer of the right heel. For this diagnosis, the home health period of care would be assigned to the 'wound' clinical group as wound care is the primary reason for the services.
4. Functional Impairment Level (subgroups of low, medium, or high)
5. Comorbidity Adjustment (subgroups of none, low, or high based on secondary diagnoses)

30 day period and case mix adjustment Payment Groups

In PDGM, a 30-day period is grouped into one subcategory under each area - admission source, timing, clinical grouping, functional impairment level, and comorbidity adjustment resulting in 432 possible case-mix adjusted groups (see below). Payment is made for each 30-day period.

Admission Source has two categories, Timing two subcategories, Clinical Grouping twelve subcategories, functional impairment level three subcategories, and comorbidity adjustment three subcategories.

$2 * 2 * 12 * 3 * 3 = 432$ case-mix adjusted payment groups

Coding Considerations

ICD-10-CM codes are used to determine the clinical group:

- 30-day period assigned to a clinical group based on the principal diagnosis code on the claim.
- The average resource use of all 30-day periods within a clinical group varies across clinical groups and the payment reflects those differences.

- If a diagnosis code is used that does not fall into a clinical group (e.g., dental codes or other uncovered/invalid codes), the claim is returned to the provider for more definitive coding.
- Additional adjustments made for other health conditions.

PDGM also has a list of diagnoses that are unspecified, vague, or not allowed to be used as principal diagnosis per ICD-10-CM Coding Guidelines and not assigned into a clinical group. If one of these diagnoses is assigned as a principal diagnosis on the patient's claim, CMS would return the claim for more specific diagnosis coding.

Some of these diagnoses are:

- M62.81 Muscle weakness, generalized
- R26.89 Other abnormalities of gait and mobility
- M54.5 Low back pain
- R26.81 Unsteadiness on feet
- R53.1 Weakness

It remains critical for coding professionals to help assure the provider's documentation is as complete and specific as possible when assigning ICD-10-CM codes in order to help prevent claim denials that delay reimbursement for the HHA.



Optional Reading: Review the "[Home Health Patient-Driven Groupings Model](#)" page provided by the Centers for Medicare & Medicaid Services for more information.

Check Your Understanding

Check your understanding of the lesson content using the practice activity below. There are six true/false questions designed to test your knowledge. When ready to submit each answer, select the checkmark submit button to verify that you are correct.

1. CPT codes are used to determine clinical groups.

☒ True

☐ False



Incorrect. ICD-10-CM Codes are used to determine clinical groups.

Continue

2. In the PDGM there are 432 possible case-mix adjusted payment groups.

☒ True

☐ False



Correct! In the PDGM there are 432 possible case-mix adjusted payment groups.

Continue

3. There are six main case-mix variables.

☒ True

☐ False



Incorrect. There are five main case-mix variables.

Continue

4. ICD-10 diagnosis and/or surgical category used to classify a SNF resident into each of the 12 clinical categories

☒ True

☐ False



Correct! ICD-10 diagnosis and/or surgical category used to classify a SNF resident into each of the 12 clinical categories.

Continue

5. The PDGM is a new payment model for the Home Health Prospective Payment System (HH PPS) became effective January 1, 2020.

☒ True

☐ False



Correct! The PDGM is a new payment model for the Home Health Prospective Payment System (HH PPS) that went into effect January 1, 2020.

Continue

6. Admission Source is a case-mix variable.

☒ True

☐ False



Correct! Admission Source is a case-mix variable.

Start Over

Lesson 3 Outpatient Prospective Payment Systems and the Ambulatory Patient Classifications (APCs)

Prospective payment systems (PPSs) were developed to help contain costs for hospital outpatient services and physician payments. The physician system – that is, the resource-based relative value scale (RBRVS) for physician services – went into effect in 1992.

The system uses work values, practice expenses, and malpractice values to calculate payment. In the hospital outpatient area, the system is called hospital outpatient prospective payment system (OPPS), with groups called ambulatory payment classifications (APCs).

In this lesson we will delve into the outpatient healthcare setting and explore their payment systems and coding considerations.

To Do:

1. Study the lesson content below
2. Complete the textbook reading as assigned
3. Check your understanding using the activity at the end of the presentation

Contemplate

Why is it important for a coding professional to understand hospital outpatient APC payment systems?

For example, consider Miss Allen, a Medicare patient, who presented to the outpatient surgery center at ABC hospital for a knee arthroscopy that included x-rays. After the coding professional, John, assigned the code(s) for the surgical arthroscopy procedure, he calculated the reimbursement for her visit. It's important for John to know that the reimbursement is paid under APCs, so *all* CPT codes assigned to Miss Allen's encounter need to be considered: those he has assigned as well as any assigned through the hospital's chargemaster that automatically flow to the account.

The Healthcare Setting: Hospital Outpatient

In 1983, Medicare moved to a PPS for hospital inpatient services to help control increasing healthcare costs and Medicare expenditures. As CMS experienced savings by reducing Medicare expenditures for inpatient care by \$17 billion per year from the payment system change, the program's administrators made efforts to incorporate prospective payment concepts into other healthcare settings. After more than 13 years of working on it, CMS implemented a hospital outpatient prospective payment system (OPPS) on August 1, 2000.

Prior to OPPS, Medicare payment for hospital outpatient services was cost-based. The cost of services was calculated by converting total charges for each encounter to cost by using department specific cost-to-charge ratios (CCRs) developed from cost report statistics. However, as healthcare costs continued to rise, CMS moved toward a prospective payment system to help encourage a more efficient delivery of care for outpatient beneficiaries.

Defining APCs

There are two types of outpatient PPS systems:

- Hospital outpatient prospective payment system (OPPS)
- Free standing ambulatory surgical center (ASC) prospective payment system

Both of these reimbursement systems are driven by the CPT®/HCPCS codes assigned.

OPPS:

The OPPS uses the groupings of ambulatory payment classifications (APCs). Each CPT/HCPCS code is assigned an APC and a corresponding payment status indicator that identifies the type of service provided. Some of these payment status indicators--such as status indicator T (for a surgical procedure) or status indicator S (for a significant procedure) result in an APC payment.

For the complete list of status indicators, see [Addendum D1](#) of the CMS CY 2021 OPPS/Ambulatory Surgery Center final rule.

APC

In the OPPS system, there are different classification types that represent the various services, procedures and supplies used in the hospital outpatient setting. The classification type for any APC can be identified by the payment status indicator assigned to the procedures and/or services in the group.

Coding Considerations

OPPS Coding

Level I (commonly referred to as CPT codes) and II HCPCS codes are the code sets used to report the services provided and any procedures performed on Medicare beneficiaries. Items and supplies are captured with HCPCS codes. Status indicator codes are assigned to HCPCS codes/APCs. The payment status indicator establishes how that service, procedure, or item is paid.

The APC Assignment

Each HCPCS code is assigned to an APC. The APC assignment for a procedure or service does not change based on the patient's medical condition or the severity of illness. It is possible to have more than one APC assigned to a single encounter.

APC Example

A patient is admitted to the ED for a possible left hip dislocation. A unilateral hip X-ray is performed and shows that the hip is dislocated, and treatment is provided to return the hip to proper alignment. The dislocation treatment is assigned to APC

group 05111, which has a payment status indicator of T (APC payment). The x-ray has a status indicator of Q1 (conditional APC payment).

This means it will be packaged as if it had a status indicator of N (packaged payment) since it is being billed on the same claim as the dislocation treatment. Since the hip X-ray is conditionally packaged and performed with a T procedure, payment is only made for APC group 05111.

ASC PPS Coding

CMS instituted a prospective payment system for ambulatory surgery centers (ASCs) in 1982 in an effort to control healthcare costs. ASCs that choose to treat Medicare beneficiaries must be state-licensed and Medicare-certified and are considered a supplier of services rather than a provider.

The facility must accept Medicare reimbursement as payment in full, which equals 80% of the total reimbursement for services provided, and then the beneficiary is responsible for paying the other 20% (HHS 2007a, 42473).

Payment rates for the ASCs are adjusted to reflect the lower cost setting provided in ASCs in comparison to hospital outpatient departments.



Required Reading: Now that you have gotten an overview of OPPS, APCs, and ASC PPS study the textbook material to learn additional details of each system. Read the subsection "Ambulatory Payment Classification System" and the "OPPS Provisions" section that follows, 7th Ed. pages 106-115 or 8th Ed. pages 136-150 in Chapter 7 of *Principles of Healthcare Reimbursement and Revenue Cycle Management*.

Check Your Understanding

Check your understanding of the lesson content using the practice activity below. There are one set of six terms to match. Drag each definition to the appropriate drop zone and use the checkmark submit button to verify your answers.

IPPS	MCR payment system where each case is categorized into a DRG which is assigned a weight, based on the average resources used to treat a patient in that DRG
RBRVS	Resource measurement system that assigns values to services performed by health professionals based on several factors
OPPS	MCR payments system for outpatient hospital services using a set of relative weights, a conversion factor, and adjustments for geographic differences in input prices
APC	Health care facilities that offer patients the opportunity to have selected surgical and procedural services performed outside the hospital setting
ASC	Outpatient hospital payment system under OPPS for Medicare patients
CCR	Calculated as a hospital's total gross charges divided by its total Medicare-allowable cost

Lesson 4: Physician Reimbursement

Physician Reimbursement

The Medicare Physician and Other Health Professional Payment System provides reimbursement to physicians and other health professionals. This payment system is divided into two parts, the resource-based relative value scale (RBRVS) and the Medicare physician fee schedule (MPFS).

To Do:

1. Study the lesson content below
2. Complete the textbook reading as assigned
3. Check your understanding using the activity at the end of the presentation

Contemplate

How is the Medicare Physician and Other Health Professional Payment System different from prospective or retrospective payment systems?

The Medicare Physician and Other Health Professional Payment System uses CPT/HCPCS codes like OPPS/APCs and ASC PPS but the reimbursement calculation varies. This is a two-part system based on resource-based relative value scale (RBRVS) and the Medicare physician fee schedule MPFS). The RBRVS is a unit of measure and similar to relative weights (which are seen in PPS).

Who, What, and Where

These are examples of the who, what and where of the Medicare Physician and Other Health Professional Payment System:

Who:

The clinicians who are paid under the system include:

- Physicians
- Audiologists
- Chiropractors
- Clinical social workers
- Nurse midwives
- Nurse practitioners
- Nutritionists
- Optometrists
- Podiatrists
- Psychologists
- Physician assistants
- Physical and occupational therapists
- Speech language pathologists

What

The services reimbursed under this system include:

- Office/clinic/emergency department visits
- Diagnostic medical and surgical procedures
- Therapeutic medical and surgical procedures
- Therapies (i.e. physical, occupational, speech)

Where

The settings where services paid under the system are provided include:

- Physician offices
- Instant-care
- Specialty clinics
- Emergency departments
- Free-standing radiology centers
- Ambulatory surgery centers
- Inpatient acute-care hospitals
- Outpatient dialysis facilities
- Skilled nursing facilities
- Hospice

Resource-based Relative Value Scale (RBRVS)

The resource-based relative value scale (RBRVS) is a federal system used to prospectively determine the cost for procedures and services to be used in calculating physician reimbursement. This system uses the relative value of the resources needed to deliver a service or product to determine its cost.

The values in the RBRVS are calculated based on three components:

1. Relative value units (RVU)
2. Geographic adjustment
3. Conversion factor (CF)

Relative Value Units:

RVUs are assigned to CPT and HCPCS Level II codes used to report procedures or services provided by physicians or other qualifying healthcare professionals.

The weight of the RVU is calculated based on the value of three elements:

- Physician work (WORK) (51% of the RVU total weight)
- Physician practice expense (PE) (45% of the RVU total weight)
- Malpractice (MP) (4% of the RVU total weight)

Geographic Practice Cost Indexes (GPCIS)

The cost of each of the three elements of an RVU is adjusted to reflect local costs through application of the geographic practice cost indexes (GPCIs).

This adjustment ensures that the cost assigned to each element is appropriate for the locality where the service or procedure is provided.

Each element of the RVU — WORK, PE, and MP — has its own unique GPCI. The GPCIs can be found on the CMS website - see their [PFS Relative Value Files](#) page.

Conversion Factor:

The conversion factor (CF) is a set amount that applies to the entire RVU. The conversion factor is the same for all geographic locations.

Application of the conversion factor transforms the geographic-adjusted RVU into a fee schedule amount. CMS has direct control on raising or lowering physician reimbursement by adjustment of the conversion factor.

RBRVS Payment Calculation

Once the RVUs are adjusted for geographic costs the components of the RVUs are summed and the total multiplied by the conversion factor to determine the national payment allowance for that procedure or service.

$(\text{WORK RVU} * \text{WORK GPCI}) + (\text{PE RVU} * \text{PE GPCI}) + (\text{MP RVU} * \text{MP GPCI}) = \text{SUM}$

$\text{SUM} * \text{Conversion Factor} = \text{National allowance.}$

Medicare Physician Fee Schedule (MPFS)

Once the RBRVS value is determined for every applicable CPT and HCPCS Level II code, the values are used to populate the Medicare Physician Fee Schedule (MPFS). The MPFS is then used to determine physician reimbursement.

Although the values in the RBRVS are determined prospectively, the MFPS is not a prospective payment system. This is because the payment made to physicians is based on the number and type of services provided; reimbursement to the physician increases proportionally to the number of procedures or services increase.

RBRVS and MPFS are not interchangeable. Remember RBRVS is a resource measurement system and MPFS is a reimbursement methodology platform.



Optional Reading: See table 8.2 on 7th Ed. page 124 or 8th Ed. page 159 of *Principles of Healthcare Reimbursement and Revenue Cycle Management* for an example.

Quality Payment Program (QPP)

The Quality Payment Program (QPP) was established by the Medicare Access and CHIP Reauthorization Act, more commonly known as MACRA. This program provides incentivized payment to physician and eligible clinicians when they fulfill specific quality measures and meet cost saving goals.

There are two ways to earn incentives under the QPP, through the Merit-Based Incentive Payment System (MIPS) or through alternative payment models (APMs).



Required Reading: To learn more about how providers can earn incentives through MIPS and APMs, read the section titled "Quality Payment Program" on 7th Ed. pages 126-129 or 8th Ed. pages 164-165 in Chapter 8 of *Principles of Healthcare Reimbursement and Revenue Cycle Management*.

Check Your Understanding

Check your understanding of the lesson content using the practice activity below. There is one screen with 6 items to categorize. You will automatically receive indication of whether you have correctly categorized each item. Once you have correctly categorized all items, use the checkmark submit button to verify and receive a feedback message.

Drag and drop each item to the box for the system or program it is associated with.

Populated by the RBRVS	Quality measures	Geographic price cost indexes
Merit-based incentive payment system	Payment increases with number of procedures	Relative value units
RBVS	MPFS	QPP

Answers:

RBVS – Geographic Price cost indexes, Relative value units

MPFS- Populated by the RBRVS, Payment increases with number of procedures

QPP-Quality Measures, Merit based incentive payment system

